Taxotere, Docivyx (docetaxel)

Effective Date: 11/10/2024
Revision Date(s): 5/23/2024, 9/4/2024, 1/17/2025, 7/1/2026
Review Date: 7/1/2026
Policy type: Medical Necessity
Line of Business: All G.E.H.A Medical Plans

Authorizations are issued for 6 (six) months, unless the ordering physician requests a different timespan or the patients' unique circumstance or condition supports the medical necessity for a different authorization timeframe. Reauthorization requests are reviewed for efficacy, safety and tolerability.

Preference	Requires Prior Auth	Drug Name	HCPCS Code	Description
Preferred	No	**Taxotere (docetaxel)**	**J9171**	Injection, docetaxel, 1 mg
Preferred	No	**Docetaxel (Hospira 505(b)(2))**	**J9232**	Injection, docetaxel (Hospira), not therapeutically equivalent to J9171, 1 mg
Non-Preferred	Yes	**Docetaxel (Docivyx)**	**J9172**	Injection, docetaxel (docivyx), 1 mg

- Members must have documentation of a contraindication, failure, or intolerance to the preferred agent prior to approval of a non-preferred product.
- Preferred product requirements apply to new starts ONLY.
- Step Therapy applies to all overlapping compendia supported indications/regimens.

Universal Approval Criteria:

Unless otherwise noted, the review criteria used by OncoHealth to determine medical necessity for anticancer treatments and supportive agents include, but is not limited to:

- National Comprehensive Cancer Network® (NCCN) - use consistent with NCCN recommendations carrying a Category 1 or 2A level of evidence; **OR**
- United States Food and Drug Administration (FDA) labeling - new drugs or regimens (combinations of drugs) consistent with all components of the product labeling; **OR**
- Indications not included in the official FDA labeling or recommended by NCCN (Category 1 or 2A level of evidence) may be considered if determined to be medically necessary per one or more of the following compendia:
    - Clinical Pharmacology (Strong For); **OR**
    - Wolters Kluwer Lexi-Drugs® (Level A); **OR**
- Other uses of drugs and biologics may be considered medically necessary if supported as safe and effective according to peer-reviewed articles from one of the following journals:
    - American Journal of Medicine; Annals of Internal Medicine; Annals of Oncology; Annals of Surgical Oncology; Biology of Blood and Marrow Transplantation; Blood; Bone Marrow Transplantation; British Journal of Cancer; British Journal of Hematology; British Medical Journal; Cancer; Clinical Cancer Research; Drugs; European Journal of Cancer (formerly the